

Behavioral & Psychiatric Disorders: Complications in Dental Treatment

OMFS 512: Integrated Biomedical and Clinical Sciences

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Mission: To advance the science and art of health through
education, service, scholarship and social accountability

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Learning Outcomes & Competency Statements

Learning Outcomes

1. Review of the selected behavioral and psychological disorders.
2. The impact of behavior and psychiatric disorders on dental treatment.
3. Review the oral manifestations associated with the disorders.
4. Learn the dental modifications needed to provide safe dental treatment in patients with behavioral and psychological disorders.

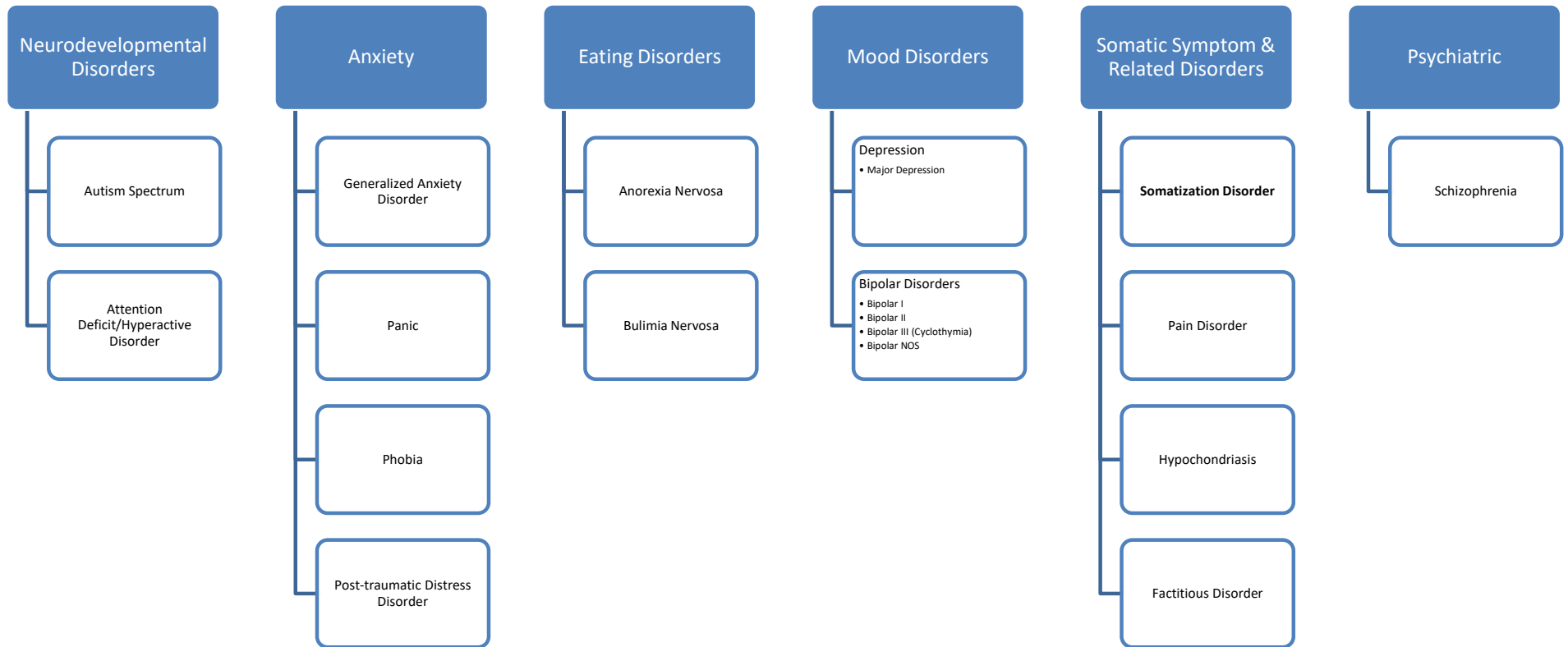
Competency statements met

#1.3, 3.1, and 5

Behavioral & Psychiatric Disorders

- One third of all Americans have at least one psychiatric disorder.
- The patients seen in dental practice can be affected by mental and behavioral health issues.
 - Significant impact on patient motivation to maintain oral health
 - Prescribed medications can affect the oral cavity
 - Behavior concerns
 - Ability to communicate and consent to treatment

Classification of Disorders



Oral Manifestation of Psychiatric Disorders

- High caries rate
- Periodontal disease
- Abrasion, hyperplasia, hyperkeratosis of the soft tissues
- TMD and myofascial pain
- Tooth wear
 - Abrasion
 - Abfraction
 - Bruxism
 - Erosion
 - Perimolysis

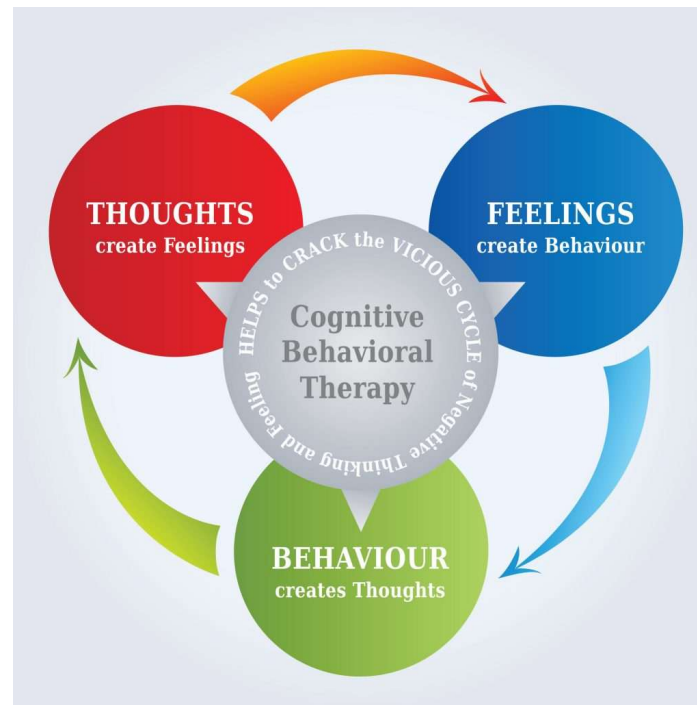


Decisions in Dentistry. March 2020;6(3):32-35



Medical Management of Psychiatric Disorders

- Pharmacological
- **Nonpharmacological therapy**
 - Cognitive therapy
 - Mindfulness
 - Relaxation techniques
 - Exposure therapy
 - Desensitization
 - Flooding
 - Stress-reduction protocol
 - Communication



Autism

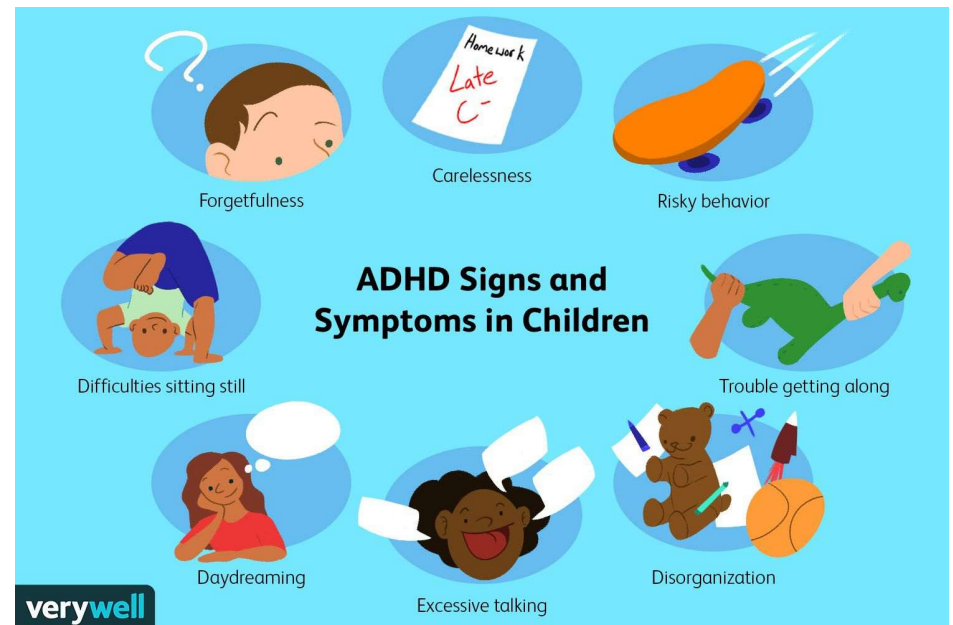
- Patient are on a spectrum from mild to more severe disease
- Epidemiology 1/40-1/500
- Slight male predilection
- Changes become evident within the first two years of life
 - Affected individuals fail to reach developmental and social milestones
 - Delays in language/speech
 - Lack of interest in socialization
 - Show narrow interest
 - Resistance to changes
 - Co-existing diseases

Dental Modification

- Discussion of the patient with the caregiver
- Desensitization
- Providing instruction with the “tell-show-do” method.
- Praise and reinforce good behavior.
- Develop a routine
- Reduce distraction
- Referral to specialists

Attention Deficit/Hyperactivity Disorders (ADHD)

- Usually diagnosed during childhood, but diagnosis may occur in adulthood.
- Behavior
 - Restlessness, fidgeting, excessive speaking, difficulty focusing, remaining seated, forgetful, disorganized, impulsive, lacks attention to detail



Management of ADHD

- Prescriptions
 - Methylphenidate
 - Amphetamines
 - SSRIs (anti-depressant)
- Modification in dental treatment
 - Early appointments
 - Offer breaks, if possible, during treatment
 - Sedation
 - Clear instructions
 - Positive reinforcement

Generalized Anxiety



Phobias

- A persistent fear of an object, situation, or activity.
 - This fear can be irrational or associated with a past traumatic experience (**Post-traumatic stress syndrome**).
 - Patients may experience a feeling of powerlessness.
 - Panic attacks

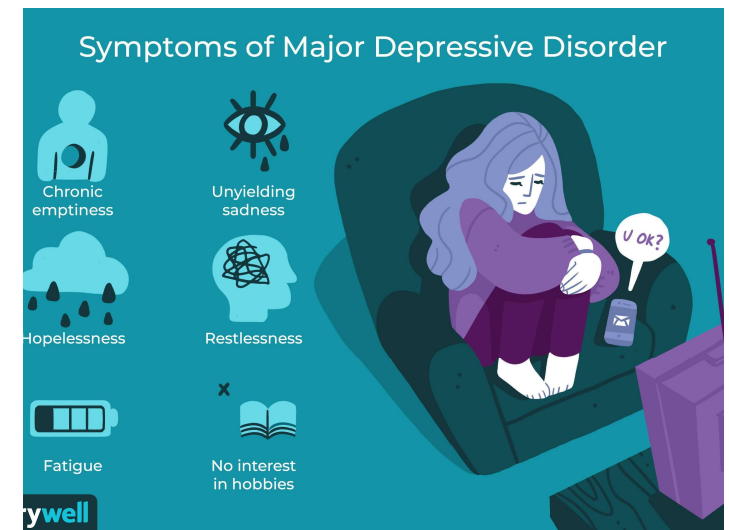
Phobias

- Phobias in Dentistry
 - White coat syndrome
 - Post-traumatic stress syndrome
 - Past unpleasant dental experience



Major Depression – Unipolar Depression

- Symptoms are present for more than 2 weeks
- Daily depression
- Lack of interest in normal activities
- Weight gain or loss
- Issue with sleep cycles
 - Hypersomnia
 - Insomnia

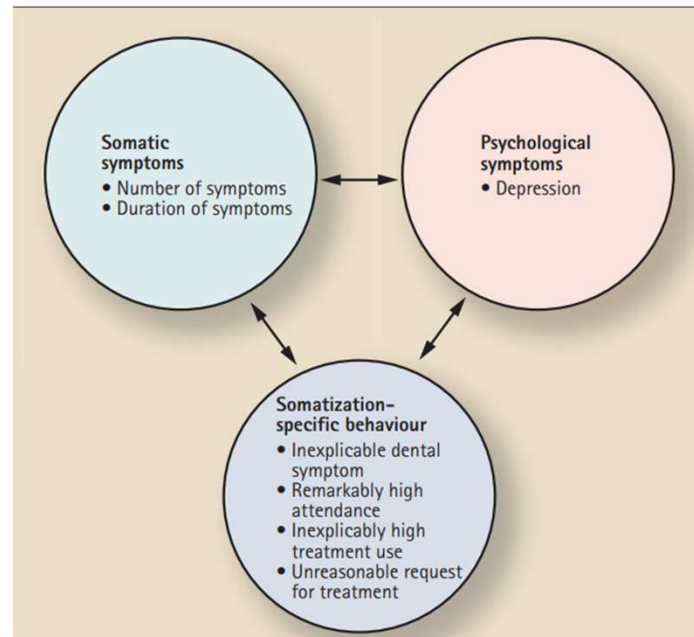


Management of Depression

- Pharmacological prescribed physicians
 - Tricyclic antidepressants (TCAs)
 - Selective Serotonin Reuptake Inhibitors (SSRIs)
 - Monoamine oxidase (MAO) inhibitors
 - Benzodiazepines
 - Antihistamines
 - Beta-adrenergic receptor antagonist

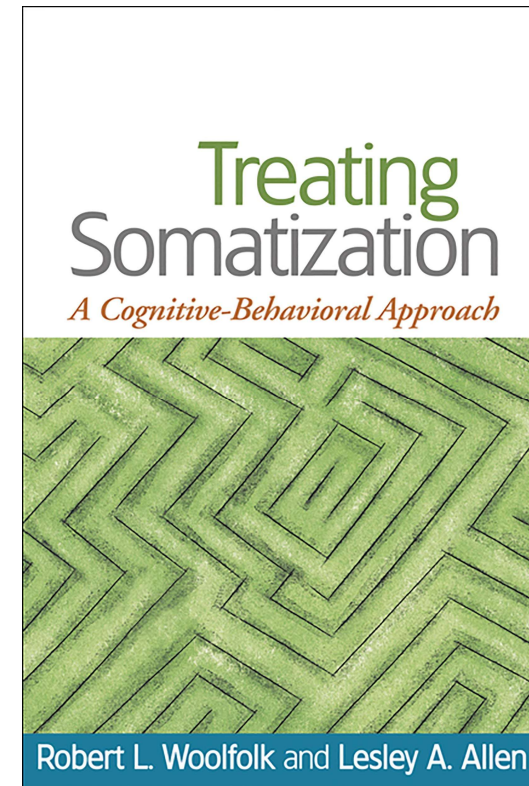
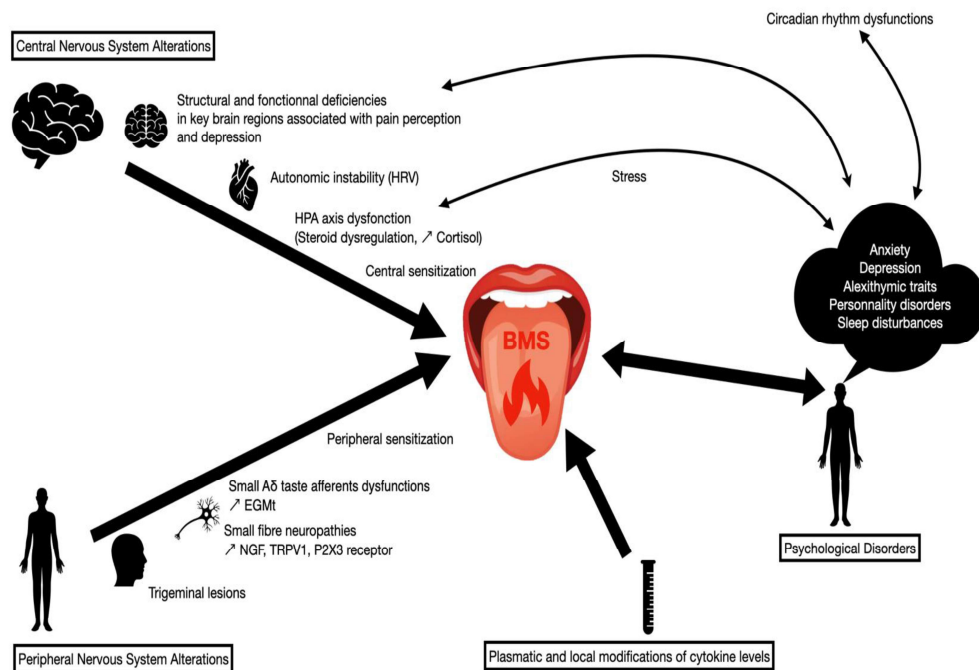
Somatization Disorder

- Patients experience unexplainable signs and symptoms of disease.
- Depression, anxiety, and stress play a role in somatization.
- Individuals are sensitive to changes in their body.



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Somatization Disorder



Bipolar Disorder

- A group of disorders in which patients experience highs (manic states) and lows (major depression).

Manic Behaviors
Inflated self-esteem
Easily distracted/agitated
Bright/colorful clothing
State of euphoria/excitable
Excessive/loud speech
Loss of social inhibition
Reduced need for sleep

BIPOLAR DISORDER

MANIC

- Onset between 18-30 yrs old
- Mood:
 - Elevated
 - Expansive
 - Irritable
- Speech:
 - Loud-Rapid
 - Punning
 - Poor Judgment
 - Clanging
 - Vulgar
- ? Wt. loss
- Grandiose delusions
- Distracted
- Hyperactive
- ↓ Need for sleep
- Inappropriate Dress
- Flight of ideas



DEPRESSIVE

- Previous manic episodes
- Feelings of
 - Worthlessness
 - Guilt
 - Hopelessness
- ↑ Anger & Irritability
- ↓ Interest in pleasure
- Negative views
- Fatigue & ↓ Energy
- ↓ Appetite
- Constipation
- Insomnia
- ↓ Libido
- Suicidal preoccupation
- May be agitated or have movement retardation

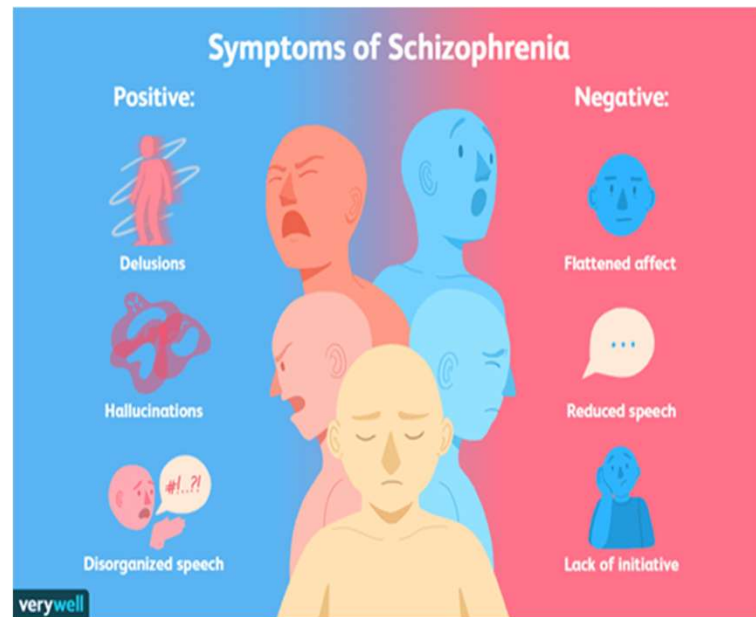
Bipolar Disorder

- Dental considerations
 - **Depends on mental state**
 - Lack of oral hygiene during depression
 - Overzealous brushing during the manic state
 - Treatment
 - Cognitive therapy
 - Mood stabilizing drugs
 - **Lithium**, valproic acid, lamotrigine, and carbamazepine.



Schizophrenia

- Psychiatric disorder caused by a combination of genetic and environmental factors.
- **Positive Symptoms**
 - Inappropriate behavior, hallucinations, and delusions
- **Negative Symptoms**
 - Withdrawal, lethargy, and a lack of communication



Schizophrenia

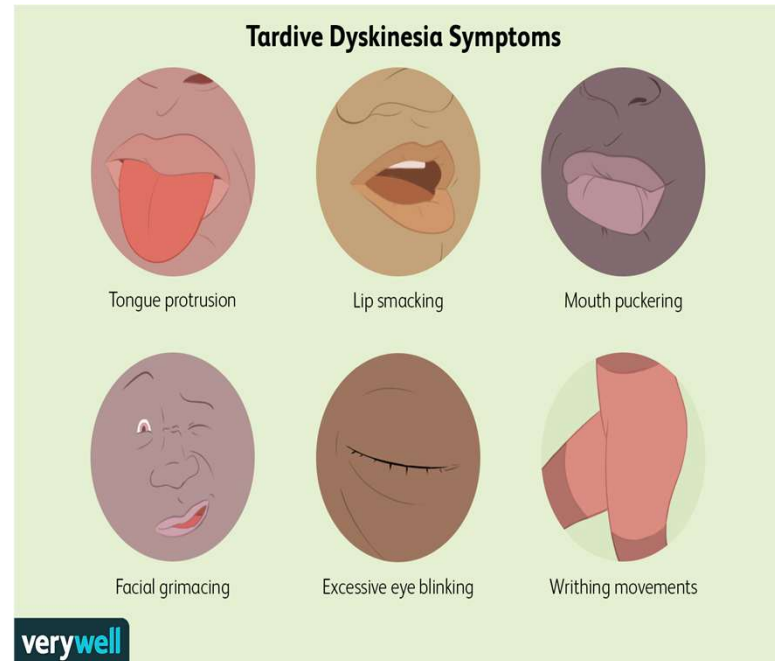
- Initial signs of schizophrenia include
 - Deterioration in self-care
 - Ability to function at work, school, and in other social settings
 - **Formal thought disorder**
 - Disorganized way of thinking leading to abnormal verbal expression
 - **Disorders of thought content**
 - Delusions
 - Hallucinations

Schizophrenia and Dental Treatment

- Behavioral concerns
- Consent
- Oral hygiene instructions
- Medication interactions and adverse reactions



Schizophrenia and Dental Treatment



Anorexia Nervosa

- Extreme fear of gaining weight
- Inability to maintain minimum weight
- Comorbidities
 - Cardiac arrhythmias, hypothermia, abnormal menstruation, malnutrition, and osteoporosis
 - Depression, obsessive compulsive behaviors, and anxiety



patientslounge.com/mental-health/An-Insight-into-Anorexia-Nervosa

Bulimia

- Alternating binge eating and purging behaviors
 - Vomiting and laxative abuse
- Maintenance of normal weight
- Comorbidities
 - Esophagitis
 - Anxiety and depression



www.steadfastnutrition.in/blogs/news/bulimia-nervosa-bn

Oral Manifestations of Eating Disorders



Periodontology 2000, 10.1111/prd.12398, 87, 1, (11-16), (2021).

California Northstate University College of Dental Medicine

Dental Considerations and Treatment

- Approach the patient in a non-judgement manner.
- Oral hygiene
 - Topical fluoride
 - Rinsing with baking soda and water
 - Brushing habits
 - Fabrication of a mouth guard
 - Discussion about nutrition

Anorexia vs Bulimia

Key Differences Between Anorexia and Bulimia

Bulimia

- Eating enormous amounts of food
- Feeling of not being able to control binges
- Forced vomiting, excessive use of laxatives or diuretics, fasting, extreme exercise
- May have a normal weight or be overweight

Anorexia

- Extreme food restrictions
- May lead to malnourishment or death
- May include exercising excessively, vomiting, or taking laxatives



The illustration depicts two individuals at a table. On the left, a woman with long brown hair, wearing a black top, sits with her hand to her face, looking distressed. In front of her is a small plate with a few pieces of food. On the right, a man with dark hair, wearing a green shirt, is eating a large slice of pizza and holding a white cup with a green straw. In front of him is a large pizza box and a bag of chips labeled 'CHIPS'. The background is a solid brown color.

verywell

Resources

Practical Oral Care for People With Autism

Waldman, HB, Wong A, Perlman P, Perlman SP, *Dimensions of Dental Hygiene*. February 2012; 10(2): 60-63

CONTENTS

HEALTH CHALLENGES IN AUTISM AND STRATEGIES FOR CARE

- 1 Communication problems and mental capabilities
Behavior problems
- 2 Unusual responses to stimuli
Unusual and unpredictable body movements
Seizures

ORAL HEALTH PROBLEMS IN AUTISM AND STRATEGIES FOR CARE

- 3 Damaging oral habits
Dental caries
Periodontal disease
 - 4 Tooth eruption
Trauma and injury
- ADDITIONAL READINGS

Providing oral care to people with autism requires adaptation of the skills you use every day. In fact, most people with mild or moderate forms of autism can be treated successfully in the general practice setting. This booklet will help you make a difference in the lives of people who need professional oral care.

Autism is a complex developmental disability that impairs communication and social, behavioral, and intellectual functioning. Some people with the disorder appear distant, aloof, or detached from other people or from their surroundings. Others do not react appropriately to common verbal and social cues, such as a parent's tone of voice or smile. Obsessive routines, repetitive behaviors, unpredictable body movements, and self-injurious behavior may all be symptoms that complicate dental care.

Autism varies widely in symptoms and severity, and some people have coexisting conditions such as intellectual disability or epilepsy. They can be among the most challenging of patients, but following the suggestions in this booklet can help make their dental treatment successful.

Making a difference in the oral health of a person with autism may go slowly at first, but determination can bring positive results and invaluable rewards.

FIFTH EDITION

Little and Falace's

DENTAL MANAGEMENT

of the Medically Compromised Patient

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